Information For the Use of
Thyroxine (levothyroxine) 62.5 mcg Tablets

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Composition

Each tablet contains:

- **Levothyroxine Sodium equivalent to Levothyroxine – 62.5 mcg**

The **62.5 mcg strength of thyroxine sodium tablets is documented in CDSCO approval records for the treatment of hypothyroidism.**

Pharmacological Action

Levothyroxine is a synthetic form of **thyroxine (T4)**, the naturally occurring thyroid hormone. It replaces or supplements thyroid hormone when endogenous production is insufficient.

It functions by:

- **Thyroid Hormone Replacement:** Provides synthetic T4 to compensate for deficient thyroid hormone production.
- **Metabolic Regulation:** After conversion to the active hormone T3 in peripheral tissues, it regulates carbohydrate, protein and lipid metabolism.
- **Growth & Development:** Supports normal growth and development, particularly in children with thyroid hormone deficiency.
- **Restoration of Euthyroid State:** Helps normalize thyroid hormone levels and associated metabolic functions in patients with hypothyroidism.

Indications

Thyine 62.5 Tablets are indicated for:

- **Primary hypothyroidism**
- **Secondary hypothyroidism** associated with pituitary disease
- **Tertiary hypothyroidism** associated with hypothalamic disease
- Congenital or acquired hypothyroidism
- **TSH suppression** as part of the management of appropriately selected patients with differentiated thyroid cancer, under specialist supervision.

Dosage and Administration

- **Recommended Dose:** Dose should be individualized according to the patient's age, body weight, cardiovascular status, clinical condition and laboratory results. **62.5 mcg once daily** may be prescribed as an individualized maintenance dose when clinically appropriate.
- **Administration:** Take once daily on an **empty stomach, 30–60 minutes before breakfast**, with a full glass of water.
- **Consistency:** Take at approximately the same time every day and maintain a consistent relationship with meals.
- **Dose Adjustment:** Dosage should be adjusted according to clinical response and periodic **TSH and/or T4** measurements. Therapeutic effects of a dose change may take several weeks to become fully apparent.
- **Duration of Use:** Treatment is generally long-term and should be continued as prescribed. Do not discontinue treatment without medical advice.

Precautions

- **Cardiovascular Disease:** Start with appropriate lower doses and titrate cautiously in elderly patients and those with underlying cardiovascular disease, as excessive thyroid hormone replacement may precipitate angina or cardiac arrhythmias.
- **Adrenal Insufficiency:** Correct untreated adrenal insufficiency before initiating levothyroxine therapy.
- **Diabetes:** Thyroid hormone replacement may alter glycaemic control; diabetic patients may require monitoring and adjustment of antidiabetic therapy.
- **Pregnancy:** Adequate thyroid hormone replacement is important during pregnancy. Dose requirements may increase, and thyroid function should be monitored regularly.
- **Bone Health:** Long-term excessive dosing may contribute to decreased bone mineral density, particularly in postmenopausal women.
- **Laboratory Monitoring:** TSH and/or T4 should be monitored periodically to ensure appropriate replacement.
- **Biotin:** Biotin-containing supplements can interfere with thyroid laboratory tests; inform the healthcare provider before thyroid-function testing.

Contraindications

- Known hypersensitivity to levothyroxine or any component of the formulation.
- **Untreated/uncorrected adrenal insufficiency.**
- Levothyroxine should not be used for weight loss or treatment of obesity in euthyroid patients.

Adverse Reactions

Common, particularly with excessive dosage:

- Palpitations
- Increased heart rate
- Nervousness or anxiety
- Tremor

- Increased sweating
- Headache
- Insomnia
- Heat intolerance
- Increased appetite or weight loss
- Muscle weakness

Rare but Serious:

- Cardiac arrhythmias
- Angina or worsening of pre-existing cardiovascular disease
- Severe hypersensitivity reactions to formulation components
- Bone loss with prolonged excessive thyroid hormone replacement

Most adverse effects associated with levothyroxine result from **over-replacement** and generally improve following appropriate dose adjustment.

Drug Interactions

- **Antacids and Calcium/Aluminium-containing products:** May reduce levothyroxine absorption. Separate administration by at least **4 hours**.
- **Iron Supplements:** Ferrous sulfate and other iron preparations can reduce absorption; separate from levothyroxine by at least 4 hours.
- **Bile Acid Sequestrants:** Cholestyramine and colesevelam can reduce levothyroxine absorption.
- **Proton Pump Inhibitors:** May affect absorption in some patients by altering gastric acidity.
- **Warfarin/Anticoagulants:** Changes in thyroid status and levothyroxine therapy may alter anticoagulant requirements; monitor appropriately.
- **Antidiabetic Medicines:** Levothyroxine may alter glycaemic control and may necessitate adjustment of antidiabetic therapy.
- **Food:** Certain foods, including soy-containing products and high-fibre diets, may affect absorption when taken close to the dose.
- Take medicines known to interfere with levothyroxine absorption at least **4 hours before or after** levothyroxine where appropriate.

Storage

- Store in a **cool, dry place**, protected from light and moisture.
- Keep the tablets in their original packaging until use.
- Avoid excessive heat.
- Keep out of reach of children.
- Follow the storage conditions specified on the approved product packaging.

Note

This information is intended as a **professional pharmaceutical-literature guideline**. **Thyine 62.5 should be used under the supervision of a qualified healthcare professional.** Levothyroxine dosage is individualized according to thyroid-function tests, age, body weight,

cardiovascular status and other clinical factors. Regular monitoring of **TSH and/or T4** is important to maintain appropriate thyroid hormone replacement.

Manufactured in India for:

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